AETNA CLINICAL POLICY BULLETIN

CPB 0860 — Direct-Acting Antivirals (DAAs) for Hepatitis C Virus

Effective 2026-01-01 · Last reviewed 2026-04 · Replaces CPB 0860 (2025-09)

§I. Scope

This policy covers prior-authorization criteria for direct-acting antiviral (DAA) therapy of chronic hepatitis C virus (HCV) infection. Drugs in scope: sofosbuvir/velpatasvir, glecaprevir/pibrentasvir, ledipasvir/sofosbuvir, sofosbuvir/velpatasvir/voxilaprevir, and elbasvir/grazoprevir.

§II. Medical necessity criteria (must meet ALL)

1. Confirmed chronic HCV infection — positive HCV RNA persisting ≥ 6 months, with documented genotype.
2. Liver disease assessment — FibroScan, FibroSure, or biopsy-staged fibrosis (METAVIR F0–F4 documented).
3. Co-infection screening — HBV (HBsAg, anti-HBc) and HIV testing completed and results documented.
4. Drug interaction screening — contraindicated medications (amiodarone, certain anticonvulsants, certain anti-retrovirals) reviewed and addressed.
5. Specialist prescriber — hepatology, gastroenterology, infectious disease, or trained mid-level under specialist supervision.

§III. Genotype-specific regimen selection (Category 1 evidence per AASLD-IDSA 2024)

Genotype 1a/1b, treatment-naive, non-cirrhotic: sofosbuvir/velpatasvir 12 wk (preferred), or glecaprevir/pibrentasvir 8 wk.
Genotype 1, compensated cirrhosis: sofosbuvir/velpatasvir 12 wk.
Genotype 2, 3, 5, 6, treatment-naive: sofosbuvir/velpatasvir 12 wk (pan-genotypic).
Genotype 4: sofosbuvir/velpatasvir 12 wk OR ledipasvir/sofosbuvir 12 wk.
Treatment-experienced or decompensated cirrhosis: defer to specialist; ribavirin augmentation may be required.

§IV. Liver transplant candidates

DAA therapy peri-transplant is medically necessary for HCV-positive transplant candidates. Pre-transplant treatment is preferred when MELD < 21 to maximize SVR rates. Post-transplant treatment requires drug-drug interaction screening with calcineurin inhibitors (tacrolimus, cyclosporine).

§V. Documentation required for adjudication

1. Genotype + viral load report.
2. FibroScan / FibroSure or biopsy result.

3. HBV / HIV serology.
4. Specialist prescriber NPI + consult note.
5. Drug-interaction screening checklist signed by prescriber.

§VI. References

AASLD-IDSA HCV Guidance (https://www.hcvguidelines.org/) · ASTRAL-1, ASTRAL-2, ASTRAL-3 trials (NEJM 2015) · FDA Epclusa label, revised 2024-09 · Aetna previous CPB 0860 (2025-09).